MAGNA-Q STUDY CRAFT SERIES

How to Use NBME Exams for USMLE Step 1

What changed when Step 1 went pass/fail, and why the goal is now to clear the line rather than to maximise

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Series: Magna-Q free study-craft guides for international medical graduates
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Study. Apply. Match. | Everything described here can be done directly with NBME and the USMLE at the published fee. This guide provides general information on published processes and statistics; it is not individual academic advice.

> **Who this is for:** anyone preparing for Step 1, written with international medical graduates in mind. Step 1 is now pass/fail, which changed what these practice exams are for — and a lot of the advice still circulating was written for the old exam.
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The thing that changed, and why most Step 1 advice is out of date

Step 1 has been reported as **pass or fail only for exams taken on or after 26 January 2022**. There is no three-digit score. There is no percentile. On test day there is one outcome and it has two possible values.

This changed your practice exams completely, and the change is more interesting than people give it credit for.

Before 2022, a self-assessment answered "what will I score?" Now it answers a different and frankly more useful question: **"what is the probability I pass?"**

That is a better question because it maps directly onto the only decision you have to make — sit the exam, or wait. And it is a question you can answer with evidence rather than feeling.

But it also means something people find hard to accept. **There is no upside to a high practice score.** If you are at 95% probability of passing, another six weeks of study buys you almost nothing, because there is no number to improve. Meanwhile a failed Step 1 sits on your transcript permanently and is visible to every residency programme you will ever apply to.

So Step 1 preparation has an asymmetry that Step 2 does not: **the entire goal is to get safely past the line and then stop.** Not to maximise. To clear, confidently, once.

What these exams actually are

The forms everyone calls "the NBMEs" for Step 1 are properly the **Comprehensive Basic Science Self-Assessment**, or CBSSA. They are made by the National Board of Medical Examiners — the same organisation that writes Step 1 — and each form has **200 items**.

That shared authorship is why they are worth more than any question bank for this specific purpose. A question bank teaches you. A CBSSA measures you, using material written by the people who will be measuring you for real.

You should also do the **free 120-question sample on usmle.org**. It costs nothing, it is official, and a startling number of people pay for an extra self-assessment while leaving the free one untouched.

There is a related exam called the **Comprehensive Basic Science Exam (CBSE)**, which some medical schools administer to their own students. It comes from the same place and reports the same way. If your school gives you one, treat the result exactly as you would a CBSSA.

Reading your score report now that Step 1 is pass/fail

Since **23 February 2022**, the CBSSA report has given you two things instead of a three-digit score:

An equated percent correct score. This is the percentage of content you have mastered, statistically adjusted for slight differences in difficulty between forms. It is on a 0–100 scale. NBME chose this deliberately, because students told them a score mattered and percent correct is a scale everyone already understands.

An estimated probability of passing Step 1 — specifically, if you were to test **within one week** of taking the self-assessment. That last part is not decoration. The estimate is calculated from a large group of examinees who actually did test within a week of a CBSSA. If you take a form and then sit Step 1 five weeks later, the number applies less well, in whichever direction your knowledge has moved.

You also get a **longitudinal report** showing your progress across multiple forms taken since February 2022, with a line indicating the average performance of a national comparison group of first-time Step 1 takers from accredited US medical schools.

That comparison line is useful context. Do not let it become a stick to beat yourself with. You are not trying to beat a national average. You are trying to pass.

A caution about the "which form is most predictive" tables

You will find rankings of the CBSSA forms all over the internet — this one is harshest, that one is most accurate, these are the best. They look like data.

NBME does not publish form-by-form predictive validity. Those rankings are assembled from Reddit threads, tutor impressions and self-reported comparisons. Some sites say so quietly in a footnote; most do not say so at all.

What is genuinely defensible is simpler: **newer forms are built from more recently retired questions**, so they reflect the current exam's style and difficulty more closely. Use newer forms for your late readiness checks for that reason — not because a table gave one a rating.

How to take one properly

Timed, one sitting, no interruptions, no looking anything up. Same time of day you have booked for the real exam.

The reason matters more for Step 1 than people think. Step 1 is a long, dense day of basic science reasoning, and the failures we see are more often concentration failures late in the day than knowledge failures. A form taken in comfortable chunks measures your knowledge. A form taken in one timed sitting measures your knowledge *and* your stamina, and both are on the exam.

Do not take one when you are ill, exhausted, or the day after a night shift. You will buy a bad number that teaches you nothing.

How to review one

The exam is the cheap part. The review is where you actually improve, and it is unpleasant enough that most people rush it.

Take every question you got wrong and sort it into one of three boxes.

"I never learned this." A real content gap. Go back to your primary resource — narrowly. The instinct after a disappointing score is to buy something new. Resist it. People who do well know one or two resources thoroughly. Adding a third resource in week nine is anxiety management wearing the costume of strategy.

"I knew this and could not retrieve it." A memory problem, not a content problem. Make a card, but make it properly: link the fact to the mechanism and to how it shows up clinically. Isolated facts on cards is how flashcard decks become 4,000 items you resent. Weight your review toward things you have now missed twice.

"I knew the material and still got it wrong." The most important box, and the one people fill in least honestly. For Step 1 this is usually one of three things: you misread what was being asked, you answered the disease rather than the mechanism, or you reasoned correctly to two options and then picked the wrong one.

For every question in this box, write one sentence saying what the question was actually testing. Then go through the wrong options and say why each is wrong. Not why the right answer is right — why the wrong ones are wrong. Different exercise, and it is the one that transfers.

Review your correct answers too. Some were guesses. A guess that landed is still a gap, and it will not land twice.

When are you ready?

Because Step 1 is pass/fail, this is cleaner than the Step 2 version of the same question — but it needs stating carefully.

The probability on your report is the number that matters. It is the only readiness figure NBME actually publishes, and it is calculated from real examinee data. Use it.

Any specific threshold you read is judgment, not published guidance — including ours. NBME does not tell you what probability means "go." Here is what we think and why, so you can argue with it:

You want a comfortable margin, on a recent form, ideally confirmed twice. A single result showing a modest probability of passing is not a green light. The whole logic of a pass/fail exam is that the downside is catastrophic and permanent while the upside is capped. That asymmetry argues for waiting until the margin is clearly wide, not until it is technically favourable.

Trend beats any single number. Three forms climbing steadily is a much better signal than one good result surrounded by weak ones. If your scores are jumping around unpredictably, something unstable is going on — pacing, fatigue, or gaps that happen to get sampled differently — and exam day will find it.

And then stop. This is the advice that is hardest to follow and most worth following. Once you are safely clear on recent forms, extra weeks of study have very little left to buy. There is no score to improve. Delaying costs you time, money, momentum and, for IMGs, sometimes a whole application cycle.

One more time, because it is the thing people get wrong: a low score on an early baseline form is normal. It is a starting point, not a prophecy. We have seen people score badly on their first form and pass comfortably. What matters is where the line ends up, not where it began.

For IMGs specifically

Register through FSMB, not ECFMG. Since **12 January 2026**, USMLE examination services for international medical graduates — registration, payment, score reports, customer service — have been provided by the Federation of State Medical Boards at usmle.fsmb.org, not through MyIntealth. A great deal of guidance online, including video walkthroughs still widely shared, describes a route that no longer exists. ECFMG still confirms your eligibility; FSMB now runs the exam.

Check your ECFMG application status before you book anything. Your Application for ECFMG Certification must show as accepted. If it was accepted while you were a student and you have since graduated, it must be updated and your credentials verified, or your exam registration will be blocked — and the reason will not be obvious from the error message.

Budget the whole sequence up front. The exam fee, the international test delivery surcharge, and your self-assessments. Deciding in advance how many forms you can afford prevents you making that decision emotionally in week eight when you are frightened.

Pacing is a language variable and it is fixable. If English is not your first language, some of your errors will be reading-speed errors on long stems rather than knowledge errors. Time yourself per block. Consistently rushing the last few questions is a pacing problem with a pacing solution, and more content review will not touch it.

Do not chase a number that no longer exists. Some IMGs still study Step 1 as though a high score will help their application. It will not — programmes see pass or fail. Your effort is better spent getting safely past Step 1 and then putting everything into Step 2 CK, which is the score they actually read.

Mistakes we see most often

- **Studying Step 1 to maximise rather than to clear.** There is no score. Get safely past and move on.
- **Taking a form untimed and then trusting the probability.** You have measured something else.
- **Hoarding the newest forms.** People save the best forms for a readiness check that keeps getting postponed. Take them.
- **Panicking over an early baseline.** It is a starting point. It is supposed to be uncomfortable.
- **Adding a new resource after a bad result.** Go narrower, not wider.
- **Treating a question bank percentage as equivalent.** Different instrument, different purpose, not interchangeable.
- **Taking your last form two days before the exam.** No time to act on it, and a bad result at that point does more harm than a good one does good.

Questions we get asked

How many should I take? At least three or four, so you can see a trend. One data point is not a trend and two is barely one.

Do these still give a three-digit score? No. Since February 2022 they report an equated percent correct score and an estimated probability of passing. Any tool still handing you a three-digit Step 1 prediction is working from a scale the USMLE retired.

My question bank percentage is much higher than my self-assessment. Why? Different instruments. A question bank measures you while you are learning, often untimed, often with hints from having just studied the topic. A self-assessment measures you cold, under exam conditions, on NBME-written questions. The self-assessment is the more honest number.

I failed a practice form. Should I postpone? Not on the strength of one form, and especially not an early one. Look at your trend across recent forms. If recent forms are still below the line, then yes, delay — a fail on your transcript is permanent, and no deadline is worth that.

Is the CBSE the same thing? Close enough for your purposes. Same organisation, same reporting since 2022. If your school administers one, use the result the same way.

Source log

Claim	Source
Step 1 reported pass/fail for exams on/after 26 January 2022	usmle.org, Examination Results and Scoring
CBSSA has 200 items; content resembles Step 1; written by NBME	nbme.org self-assessment documentation
Score reports show equated percent correct plus estimated probability of passing if testing within one week	NBME score report guidance for examinees, February 2022 onward
Probability model based on examinees who tested within one week of a first-time Step 1 attempt	NBME, Common Questions on CBSSA Score Report Changes
Longitudinal report covers forms taken on or after 23 February 2022, with a national comparison line	NBME score report documentation
CBSSA bundle discount window, 9 February – 3 April 2026	nbme.org news, 2026
USMLE services for IMGs moved to FSMB on 12 January 2026	ECFMG MyIntealth FAQs; usmle.org service transition materials

Flags

- **Readiness thresholds here are our judgment.** NBME publishes the probability estimate but no guidance on what probability constitutes readiness.
- **Form-by-form predictiveness rankings online are community-sourced**, not NBME data. We have not published one.

- **NBME retires and replaces forms periodically.** Check which forms are currently available on MyNBME rather than relying on a form number you read in an article.
- **Self-assessment pricing and bundle windows change annually.** Confirm on nbme.org.
- Re-verify this article by **1 December 2026**.

Everything here can be done directly with NBME and the USMLE at the published fee. No course or tutor is required to use these exams well — only the discipline to take them properly, review them honestly, and stop once you are safely across the line.